ISSUED, VALID FROM 30 SEPTEMBER 2026

Typhoid (Vi Polysaccharide Vaccine)

Patient Group Direction, version 008, issued 30 September 2026. Standalone document.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Scope of this PGD

This PGD authorises the administration of Vi polysaccharide typhoid vaccine for travellers to areas where typhoid is a risk.

- **This is now a standalone document. Hepatitis A and cholera are covered by their own documents.**
- **This PGD covers the INJECTABLE Vi polysaccharide vaccine only. It does not cover oral live typhoid vaccine (Ty21a, Vivotif), which is a live vaccine with entirely different handling, contraindications and interval rules. If your pharmacy stocks the oral vaccine it is not covered here.**
- Observe every patient for 15 minutes after vaccination.
- **Vaccine efficacy is roughly 70 to 80%. Food and water hygiene advice is not optional and must be given and recorded in every case.**
- The vaccine does not protect against paratyphoid A or B.

Typhoid Vi polysaccharide vaccine

Summary of Green Book chapter 33 guidance for typhoid

UKHSA, Immunisation Against Infectious Disease, chapter 33, Typhoid. Chapter footer dated 4 February 2022; GOV.UK page records the last update as 3 April 2020. Both dates are given because they differ. Summarised 9 September 2026.

Overview

Typhoid is a systemic infection caused by Salmonella enterica serotype typhi. Paratyphoid is clinically similar and is caused by S. paratyphi A, B and C.

Transmission is mainly oral, from food or water contaminated by the faeces, and occasionally urine, of cases or chronic carriers. Direct faecal-oral spread also occurs.

Incubation averages 10 to 20 days, with a range of 3 to 56.

Case fatality is under 1 per cent with prompt antibiotics, but may be as high as 20 per cent untreated or treated with the wrong antibiotic. Antibiotic resistance is increasing, including extensively drug-resistant typhoid in Pakistan.

THERE IS NO VACCINE FOR PARATYPHOID. Say so to travellers, who often assume otherwise.

The attack rate for travellers to the Indian subcontinent is estimated at 1 to 10 per 100,000 journeys.

Who the chapter recommends it for

Travellers to typhoid-endemic areas whose planned activities put them at higher risk. Check TravelHealthPro or Travax for the destination.

Those at increased risk include travellers visiting friends and relatives, and frequent or long-stay travellers to areas where sanitation and food hygiene are likely to be poor.

Laboratory personnel who may handle S. typhi.

The chapter states that typhoid vaccine is NOT recommended for close contacts of cases or carriers, nor during a UK outbreak.

Schedule, Vi polysaccharide vaccine

A single 0.5 mL dose for adults and children over two years. Each dose contains 25 micrograms of Vi antigen.

A single reinforcing dose at THREE YEAR intervals for those who remain at risk. Someone who has had a non-Vi typhoid vaccine may have Vi reinforcing doses on the same three-yearly basis.

Children aged 12 months to two years should be immunised OFF LICENCE only if a detailed risk assessment finds the risk of typhoid high. Immunisation is not recommended under one year.

A four-fold antibody rise is detectable seven days after the primary dose, maximal at one month, persisting about three years. Additional doses do not boost levels further.

Give intramuscularly into the upper arm or anterolateral thigh. INTRADERMAL INJECTION MAY CAUSE A SEVERE LOCAL REACTION AND MUST BE AVOIDED. Deep subcutaneous for a bleeding disorder. Never intravenously.

Contraindications and precautions

Do not give Vi vaccine to anyone who has had confirmed anaphylaxis to a Vi antigen-containing vaccine.

A severe reaction to a previous NON-Vi typhoid vaccine does not contraindicate a Vi-containing vaccine.

Minor illness without fever or systemic upset is not a reason to postpone. If acutely unwell, postpone until recovered.

The gastrointestinal illness, antibiotic and antimalarial timing precautions in this chapter apply to the ORAL Ty21a vaccine only, not to Vi.

Pregnancy and immunosuppression

PREGNANCY: no data are available on Vi in pregnancy or lactation. There is no evidence of risk from inactivated vaccines or toxoids in pregnancy or breastfeeding.

IMMUNOSUPPRESSION: Vi contains no live organisms and may be given to people with HIV and to those who are immunosuppressed, in the absence of contraindications. The response may be sub-optimal, so emphasise personal, food and water hygiene.

What to tell every traveller

The chapter is explicit: not all recipients of typhoid vaccine will be protected, and travellers must be advised to take all necessary precautions to avoid contact with or ingestion of potentially contaminated food and water.

The vaccine is an addition to food and water precautions, not a replacement for them.

Patient Group Direction for the administration of Vi polysaccharide typhoid vaccine (Typhim Vi or equivalent) for travellers aged 2 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. Typhoid Vi vaccine is not a controlled drug, so registered pharmacy technicians may lawfully supply and administer under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- All users must be trained to the National Minimum Standards and Core Curriculum for Immunisation Training, and be competent in intramuscular vaccination technique.
- All users must be competent in the recognition and immediate management of anaphylaxis, and hold current basic life support training.
- All users must be competent in destination risk assessment using current NaTHNaC / TravelHealthPro country recommendations.
- All users must be competent in assessing consent in children and young people, including parental responsibility and Gillick competence.
- All users must be competent in vaccine handling and cold chain management.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.

The PGD

Indication	Active immunisation against typhoid fever caused by Salmonella Typhi, for travellers aged 2 years and over going to areas where typhoid is a risk, in accordance with Green Book chapter 33 and current NaTHNaC / TravelHealthPro country guidance.
Inclusion criteria	● **Aged 2 years and over.** ● Travelling to an area where typhoid vaccination is recommended, established from current NaTHNaC / TravelHealthPro guidance and recorded. ● At least 2 weeks before departure, so that protection can develop. Where travel is sooner, vaccination may still be given but the traveller must be told protection may be incomplete, and that must be recorded. ● Valid informed consent obtained. Where the patient is under 16, from a person with parental responsibility, or from the young person where assessed as Gillick competent, with the basis of the assessment recorded. ● No exclusion criterion present.
Exclusion criteria	**Refer, do not vaccinate, where any of the following applies.** ● Under 2 years of age. The Vi polysaccharide vaccine produces a poor response below 2 years. Refer. ● Confirmed anaphylactic reaction to a previous dose of typhoid vaccine, or to any component of the product held. ● Acute severe febrile illness. Postpone until recovered. A minor illness without fever is not a reason to defer.

	• A dose of typhoid Vi vaccine within the last 3 years. Record the previous dose date; the next dose is due 3 years after it. • **Any fever following recent travel to a typhoid risk area. Typhoid is a notifiable illness and a febrile returning traveller needs urgent assessment, not vaccination.** • Under 16 with no person with parental responsibility available to consent, and not assessed as Gillick competent.
Cautions	• Pregnancy and breastfeeding. This is an inactivated polysaccharide vaccine and may be given where the risk of typhoid is significant and travel is unavoidable. Discuss and record the decision. • Immunosuppression: the vaccine may be given but the response may be reduced. Advise that protection may be lower and that food and water hygiene matters more, not less. • Bleeding disorders or anticoagulation: give intramuscularly with a fine needle (23 gauge or finer) and apply firm pressure without rubbing for at least 2 minutes (Green Book chapter 4). Deep subcutaneous injection is the alternative where the individual's haematologist or specialist advises it. • The vaccine does not protect against paratyphoid A or B, which cause a clinically similar illness. Say so.
Actions if the patient is excluded or declines	Explain why vaccination cannot be given and what the alternative is. Where the patient is under 2, refer to a travel clinic or the GP. Where there is fever following travel to a risk area, refer for urgent same-day assessment and say plainly that typhoid must be excluded. In every case give food and water hygiene advice, which is the main protection regardless of vaccination status. Document the advice and the decision.

The medicine

Name, form and strength	Typhoid Vi polysaccharide vaccine, 25 micrograms in 0.5 mL, solution for injection in a pre-filled syringe (Typhim Vi or equivalent Vi polysaccharide product).
Legal category	POM.
Route and method	Intramuscular injection, 0.5 mL, into the deltoid (every patient under this PGD is aged 2 years or over). Do not give intravascularly.
Dose and frequency	A single 0.5 mL dose, given at least 2 weeks before travel.
Booster	A booster dose every 3 years for those with continued or repeated exposure. Record the date the next booster is due.
Quantity	One dose per patient per attendance. This PGD does not permit supply of vaccine to the patient for administration elsewhere.
Adverse effects	Very common: injection site pain, redness and swelling. Common: malaise, headache, myalgia, mild fever. Uncommon: nausea, abdominal pain. Rare: urticaria, Guillain-Barre syndrome, anaphylaxis.
Anaphylaxis and observation	**Adrenaline (epinephrine) 1 in 1,000 injection must be immediately available whenever a vaccine is administered under this PGD, together with a written anaphylaxis protocol consistent with current Resuscitation Council UK guidance and a telephone.** All staff administering vaccines must be trained in the recognition and immediate management of anaphylaxis and must hold current basic life support training.

	Observe every patient for 15 minutes after vaccination. Vaccinate seated.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given, and from whom. Where the patient is a child, the person with parental responsibility or the basis of the Gillick assessment. • Patient name, address, date of birth, and the GP with whom they are registered. • Name and brand of vaccine, batch number and expiry date. • Date of administration, dose, route and anatomical site used. • Destination, itinerary and departure date, and the source consulted for the recommendation. • The date any previous typhoid dose was given, where known, and the date the next booster is due. • Name and registration number of the healthcare professional administering. • Advice given, including food and water hygiene advice and the post-travel fever warning. • Details of any adverse drug reactions and the actions taken. • That the vaccine was administered under this PGD. Records signed, dated, legible and contemporaneous. Adults: 8 years. Under 18s: until the 25th birthday, or the 26th where the individual was 17 when the dose was given.
Storage and cold chain	Store at +2C to +8C in the original packaging to protect from light. Do not freeze; discard if frozen. **COLD CHAIN EXCURSION: vaccine exposed to conditions outside the stated range must be quarantined and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use. Do not administer excursion stock until that assessment is complete.**
Disposal	Dispose of used syringes and needles in a UN-approved puncture-resistant sharps container in accordance with local arrangements and HTM 07-01.
Co-administration	May be given at the same time as other travel vaccines. Use a separate limb where possible, or sites at least 2.5 cm apart in the same limb. Record the site used for each vaccine.

Information for the patient

Written information	Supply the patient information leaflet for the product given, and the Get Real Health food and water hygiene sheet.
Counselling	• **This vaccine is about 70 to 80% effective. It reduces your risk; it does not remove it.** • **Food and water care is your main protection. Boil it, cook it, peel it, or leave it. Avoid ice, salads washed in local water, unpasteurised dairy and food from street stalls that is not cooked in front of you.** • It does not protect against paratyphoid, which causes a very similar illness. • It takes about 2 weeks to work, so it is worth having in good time before you go. • **If you develop a fever, headache and feel increasingly unwell during or after your trip, see a doctor and tell them where you**

	have been. Typhoid can present weeks after you return. ● A sore arm, mild fever and feeling off for a day or two afterwards is common and settles by itself. ● If you travel to risk areas regularly, you need a booster every 3 years.
Follow-up	No routine follow-up. Seek medical attention for fever during or after travel and disclose the travel history. Return in 3 years for a booster if you continue to travel to risk areas.

Appendix 1 , Food and water hygiene advice

Give this in every case, whether or not the traveller is vaccinated. The vaccine is roughly 70 to 80% effective and does not cover paratyphoid.

- Drink bottled water with an intact seal, or water that has been boiled. Avoid ice.
- Avoid salads and uncooked vegetables washed in local water.
- Eat food that is thoroughly cooked and still hot. Avoid food that has been standing.
- Peel fruit yourself.
- Avoid unpasteurised milk and dairy products.
- Wash hands before eating, with soap or an alcohol-based gel.

Appendix 2 , The post-travel fever warning

Typhoid has an incubation period of one to three weeks and can present after the traveller is home. Any fever during or after travel to a risk area should prompt medical assessment with the travel history disclosed.

Appendix 3 , What this PGD does not cover

- **Oral live typhoid vaccine (Ty21a, Vivotif). It is a live vaccine with different contraindications, different interval rules with other live vaccines and antibiotics, and different storage. It is not covered here.**
- Children under 2 years.
- Treatment of suspected typhoid. That is a medical emergency.
- Hepatitis A and cholera, which have their own documents.

Authorisation

Version	v008
Supersedes	Version 007, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

Change history to version 003

Later versions are recorded in the Version and change record at the end of this document.

Version	v003
Date	7 September 2026
Changes	● The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. ● Full clinical review and reissue as a standalone document.

Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....

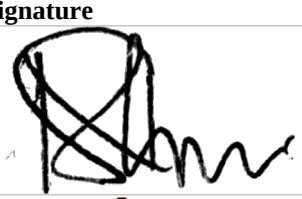
Version and change record
Version: v008
Issued: 30 September 2026
Supersedes: Version 007, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.
This PGD's staff section did not carry the requirement, common to every other Get Real Health PGD, that users have completed training in working under Patient Group Directions and been assessed as competent to work under a PGD before first working under this one. It is added, with the training-before-first-use statement.

Previous version records
Version: v007
Issued: 14 September 2026
Supersedes: Version 006, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v006
Issued: 11 September 2026
Supersedes: Version 005, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
The 3 year exclusion no longer carries the circular exception about a dose 'due for renewal', which could never be satisfied inside the 3 year window; a dose within the last 3 years excludes.
The bleeding disorder caution now states the intramuscular fine needle technique with the Green Book chapter 4 reference and names deep subcutaneous injection as the alternative on specialist advice, so it agrees with the guidance summary.
The route row states the deltoid without an age qualifier that excluded nobody.
The retention row now carries the 26th birthday clause used in the other PGDs.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v005
Issued: 11 September 2026
Supersedes: Version 004, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.
Previous version records
Version: v004

Issued: 10 September 2026
Supersedes: Version 003, 9 September 2026
Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.

Signed on behalf of Get Real Health

Version v008, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.